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Document Zero

The Foundation



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What the new neuroscience means for every practitioner.

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About the Author

Charmian Berry holds a psychology degree and a counselling diploma, and worked as an assistant clinical psychologist before training as an accredited CBT therapist. She trained in EMDR, completed IFS training directly with Dick Schwartz, studied CRM with Lisa Schwartz, and trained in polyvagal-informed practice with Deb Dana. She was co-founder of the Manchester Monastery Listening Service — a trauma-informed community mental health project that broke genuinely new ground in accessible psychological support.

She played a significant role in introducing somatic trauma work, IFS and polyvagal theory to the UK plant medicine community over a decade ago. She has spent the years since as a nondual enquirer, sitting with the question that survived every therapeutic model she trained in: what actually ends suffering?

That question — and the convergence of predictive processing neuroscience with direct inquiry — became The Liberté Code.

She is the author of The Liberté Code: How the Predictive Mind Creates Suffering (2025) and creator of the TLC Practitioner Series and Decoder — bringing neuroscience upgrades and nondual understanding to the practitioner and wellbeing field.

The Foundation

Something has shifted in the sciences of mind over the past three decades, and it hasn't yet fully reached the consulting room.

Not because practitioners aren't curious — the people drawn to this work tend to be among the most intellectually engaged and personally committed in any profession. But the research is technical, the implications are wide, and nobody has yet done the work of translating it into the language of clinical and healing practice in a form that is both honest and immediately useful.

That is what this document is for.

What follows is not a critique of the frameworks you've trained in, or of the years you've invested in developing genuine skill and sensitivity. It's something more interesting than that. It's a clarification — of what those frameworks are actually doing when they work, why they sometimes hit a ceiling they can't account for, and what becomes possible when the mechanism underneath them is understood precisely.

The predictive processing model of mind — developed over the past thirty years by researchers including Karl Friston, Andy Clark, and Lisa Feldman Barrett — has produced a fundamentally new account of how the brain generates experience, how suffering arises, and what actually resolves it. This account doesn't sit comfortably alongside some of the models most practitioners trained in. It sits beneath them, examining their foundations, and finding that several of those foundations rest on assumptions that the neuroscience no longer supports.

This matters not as an academic argument but as a practical one. Because the ceiling most practitioners eventually encounter — the client who returns year after year, the pattern that reconstitutes itself after every breakthrough, the insight that lands and then quietly dissolves — is, in most cases, a ceiling built into the model itself. Understanding where it is, and why, is the first step toward working past it.

What the Brain Is Actually Doing

The assumption that has quietly shaped almost every therapeutic and healing approach of the past century is this: that the mind works primarily by recording experience, and that suffering arises when those recordings — particularly the difficult ones — remain unprocessed, unintegrated, or stored somewhere in the body and nervous system, pressing for expression.

It is an intuitive model. It maps onto how memory feels, and how symptom relief often seems to follow when something buried is finally brought to light. And it has produced genuine advances — it moved the field away from blame, toward compassion, toward the body, toward an understanding that what people experience has causes that deserve to be taken seriously.

But the neuroscience is now showing something different about how the brain actually operates — and the difference is significant enough to change the entire picture of where suffering lives and how it resolves.

The brain is not primarily a recording device. It is a prediction engine.

At every moment, the brain is generating a model of what it expects to encounter — drawing on everything it has previously learned to anticipate what the next moment will bring. Incoming sensory data doesn't build experience from scratch; it is used to check and refine the predictions already running. Experience, in other words, is not what arrives from outside. It is what the brain generates in advance, continuously, and then adjusts at the margins.

If experience is generated in the present — constructed now, by a system running its best current model of what this moment means — then what a person feels is not a stored memory surfacing from somewhere else. It is a prediction being made right now, shaped by prior learning but entirely current in its operation. The anxiety is not old fear leaking into the present. It is the brain predicting threat in this moment, based on a model it built from past experience. The shame is not a residue of what was done to someone decades ago. It is the brain's current generation of a familiar experience, because nothing has yet arrived that would give it reason to generate something different.

This shifts the location of suffering from the past to the present — which is actually good news, because the present is where it can be worked with directly. It also shifts the question from what happened and what did it leave behind, to what is the system predicting now, and what would update it. These are different inquiries, and they lead to different work.

What This Means for the Modalities

None of this makes the modalities wrong. EMDR produces genuine and lasting change. IFS reaches places that talk therapy rarely touches. Somatic work, breathwork, constellation work — these approaches access something real, and skilled practitioners within all of them witness shifts that are difficult to explain within a purely cognitive framework.

What predictive processing provides is a more precise account of what these approaches are actually doing when they work — and why they sometimes don't.

Every modality that produces genuine, lasting change is doing the same three things, whether it knows it or not. It is providing a context — a frame that makes the experience intelligible and safe enough for the system to risk something new. It is creating permission — an internal signal, however it's established, that it is safe to let an old prediction go. And it is delivering new information — something that doesn't confirm what the system already expects, landing clearly enough and safely enough to register as genuinely different.

> *Context. Permission. New information. When all three are in place, the prior updates. The change is often subtle — not cathartic, not dramatic — but it holds, because it is structural rather than surface.*

When they aren't all in place, the work produces something that looks like change but doesn't last. The insight lands intellectually but doesn't reach the level where the prediction is being generated. The release happens in the room and reconstitutes by Thursday. The breakthrough is followed, sometimes within days, by a return to the familiar pattern — what looks like regression, or backlash.

This is not failure. It is the system doing exactly what it was designed to do — maintaining coherence, restoring what was structural, re-establishing the predictions that organise its model of self and world. Understanding this removes a significant weight from practice. The system isn't resisting. It is being consistent. And once you can see what it's being consistent about, the work becomes considerably more precise.

The practical implication is this: you don't need the trauma storage model to do somatic work effectively. The body responds to predictions, and when the prediction changes, the body changes with it — because the body was always expressing the prediction, not storing the wound. You don't need parts to be semi-autonomous beings to work with IFS effectively. The parts are coherent prediction clusters, each organised around a foundational prior, and understanding them as such makes the work more precise, not less. You don't need ancestral transmission to be literally genetic to work meaningfully with lineage patterns. The deepest

priors in the system are species-level — they predate biography entirely — and working at that level is genuinely profound, whatever story is or isn't wrapped around it.

What changes when you understand the mechanism is not what you do. It is the understanding you bring to what you do — and that understanding, it turns out, matters considerably more than practitioners are usually taught to think.

Four Assumptions Worth Examining

Every therapeutic model, every healing framework, every practitioner training — however different they look on the surface — rests on a set of foundational assumptions about how suffering works. Most practitioners absorbed these assumptions so early and so completely that they stopped seeing them as assumptions. They became invisible ground.

Here they are, made visible. And here is what the neuroscience now shows about each one.

Assumption One: Something is broken

Something went wrong. The client arrives damaged, dysregulated, traumatised, stuck. The work is to fix, heal, restore, regulate, integrate. The implicit message — however gently delivered — is: you are not okay as you are.

The neuroscience says: *Nothing is broken at the level of the generative system. The system is working exactly as designed — generating predictions based on the model it has built from experience. The predictions may be inaccurate, narrow, or painful. The system generating them is not defective. There is nothing to fix. There is something to see.*

Assumption Two: The past is the cause

The suffering happening now is because of something that happened then. Childhood wounding. Developmental trauma. Ancestral patterns. The story of how we got here is located in a past the client may not even consciously remember.

The neuroscience says: *The past is not stored in a form that can be accessed and released. Memory is reconstructive — rebuilt in the present moment from available information. What we call the past is a current-moment generation. The suffering is happening now, in a generative system running predictions learned from past experience. The location of suffering is always present.*

Assumption Three: It is held somewhere inside

Whatever happened left something lodged in the body, the nervous system, the energy field, the cellular memory. It is stored. Waiting. Pressing for expression. Creating symptoms until it is finally released.

The neuroscience says: *Nothing is trapped. The body is not a repository of stored experience — it is a rendering device, expressing the brain's current predictions in real time. The tension, the activation, the pain are current-moment expressions of current predictions. There is nothing to excavate. There is a prediction to understand.*

Assumption Four: The work is to get it out

The work is extraction. Release. Processing. Integration. Moving through. All variations on the same premise: there is something in there that needs to come out before the person can be free.

The neuroscience says: *Freedom does not come from extraction. It comes from update. When the generative model receives sufficient disconfirming information — when the prediction is not confirmed by experience — the prior updates. This is not release. It is revision. The model learns something different, and generates a different experience.*

These assumptions have shaped virtually every therapeutic and healing modality of the past century. They have produced genuinely useful frameworks and helped millions of people. They made sense given what we knew.

The neuroscience is now showing us that all four rest on a model of mind that is no longer well-supported. And the good news — genuinely good news — is that the more precise model is not more complicated. It is simpler. It points at something that was always here, before the story, before the wound, before the framework that said there was something to fix.

The Architecture Underneath

The predictive system doesn't only generate moment-to-moment experience. It generates a self — a continuous, coherent identity with a history that explains the present and a future that needs to be secured. And that self is built on structure.

In the TLC framework, this structure is called the Load-Bearing Walls.

These are not beliefs that can be challenged in a CBT session, or parts that can be unburdened, or schemas that can be updated through corrective emotional experience alone. They are the foundational templates the predictive system constructed — usually very early, always for reasons that made complete sense given the conditions — to answer the most urgent questions a developing human faces. Questions about safety, belonging, worth, and what is required to remain connected to the people on whom survival depends.

They are load-bearing because the entire identity is built on top of them. They don't present as walls — they present as ground. They feel like simply how things are, rather than a structure that was built and could, in principle, be seen through. And when something touches one of them without that seeing occurring, the system doesn't open. It restores. This is the mechanism behind every backlash, every return to baseline after a breakthrough, every client who seems to understand everything and still cannot shift.

There are eight walls. Each one organises a particular cluster of what the TLC framework calls hidden desires — the payoffs the system is running underneath the presenting problem, the reasons the prediction persists even when the person consciously wants it to change. Each wall is readable in the room, once you know what you are looking at. The stuck case is almost always stuck at the level of the wall, not at the level of the content being worked with.

The TLC Practitioner Series maps each wall in clinical detail, modality by modality, with direct application to the work you are already doing. The TLC Decoder reads this architecture in real time, underneath whatever a client is presenting — identifying the walls, the hidden desires sustaining them, and the update conditions most likely to create genuine movement.

Where the Neuroscience Leads

There is one further step the predictive processing model takes — one that most of its clinical applications stop short of, but that the framework itself makes unavoidable if you follow it honestly.

The self that the Load-Bearing Walls are protecting is itself a prediction.

Karl Friston, whose free energy principle provides the mathematical foundations of predictive processing, has followed his own model to this conclusion. The sense of being a continuous, bounded, authored self — the one to whom all of this is happening, the one whose history explains the present and whose future needs securing — is not a fixed reality the brain perceives. It is something the brain generates, just as it generates everything else. The self-model is the most protected prediction of all, which is why it is the last thing any therapeutic framework tends to address, and why the work so often reaches a ceiling precisely there.

This is where every contemplative tradition worth paying attention to has been pointing for centuries, in languages and methodologies that have nothing to do with neuroscience. The Zen practitioner and the nondual inquiry facilitator and Karl Friston are, from completely different angles, arriving at the same recognition: that the one who appears to be suffering is itself part of what the system is generating.

This is not a philosophical position to be adopted or a spiritual framework to be trusted or distrusted. It is something that can be looked at directly — and when it is, the solidity that the whole structure of suffering depends upon turns out to be less fixed than it appeared.

In the TLC framework this is called Wall 0. It is the foundational prior — not a wound, not a schema, not a part — and it is what every other wall was built to protect. When it is seen clearly, not as a concept but as a direct recognition of what is actually here before the story begins, something shifts that no technique produces and no framework fully accounts for.

> *What TLC is doing, at its deepest level, is building a bridge between the Western therapeutic model and that recognition. A bridge that doesn't require abandoning clinical language, or leaping into a spiritual framework, or accepting anything on faith.*

One that follows the neuroscience — all the way, without stopping where it becomes uncomfortable — and arrives somewhere that the best practitioners have always been circling without quite having the map to reach.

A Note on Love

The question sometimes arises — where is the compassion in a framework that questions the wound?

It is worth answering directly, because it touches something important about what this work is for.

The most common assumption in therapeutic and healing practice is that compassion means validating the story — meeting the person's account of their damage with warmth, acknowledgement, and confirmation. And that quality of meeting is genuinely valuable. It is often the first time someone has felt truly heard, and that matters enormously.

But there is a more precise compassion available when you understand the mechanism. One that can fully acknowledge the reality of someone's experience — the sensation, the emotion, the felt sense of what they carry — without confirming the narrative that has formed around it as the final truth of who they are. Because the narrative, however understandable, is also part of what the system is generating. And tending it indefinitely, however lovingly, is not the same as helping someone find their way to what is actually here before the generation starts.

The practitioner who holds someone in their actual wholeness — not their story of brokenness — is doing something more demanding, and more loving, than the one who validates every layer of the wound. It requires genuine conviction that there is nothing fundamentally wrong with the person sitting across from you. Not as a technique. As something actually believed.

That is what the predictive processing model, followed all the way down, makes possible. Not a colder practice, but a more honest one. Not less compassionate — more precisely so.

What's Available From Here

Nothing here needs to be taken on as a belief, adopted as a new identity, or defended against your existing practice. If something in these pages has landed — even lightly, even as a question rather than a conclusion — that is enough to begin. The system responds to what it encounters. So do we.

For those who want to go further, there is a clear path.

The TLC Practitioner Series

Nine documents, each written for a specific modality — IFS, EMDR, somatic work, breathwork, CBT, psychotherapy, plant medicine and non-ordinary states, energy healing, and ACIM. Each takes the framework you already work within and shows precisely where the predictive processing model clarifies, deepens, and occasionally corrects what the modality has been pointing at — with direct clinical application throughout.

The TLC Practitioner Decoder

A clinical AI tool built on this framework. Bring a client presentation and the Decoder maps the predictive architecture underneath it — the load-bearing walls, the hidden desires sustaining them, the update conditions most likely to create genuine movement. Practitioners describe it as having a thinking partner who has read everything, carries no agenda, and can see the structure of a presentation without being pulled into its content.

The Decoder Circle

A live weekly community of practitioners working within this framework, bringing cases, exploring the edges, and supporting each other through the shift this understanding tends to produce. Because the framework doesn't only change how you work with clients. It begins to work on you. And having company in that is worth something.

The Liberté Code

The book — where this all began and the most complete single account of the framework, the evidence, and the clinical and philosophical case for following the neuroscience all the way down.

Charmian Berry

Creator, The Liberté Code

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The therapeutic world has been asking: how do we make the self better?

The revolution begins when we ask: what is the self?

And when we find that question has an answer — and the answer changes everything.

The end of unnecessary suffering is not a dream.

It is the natural consequence of a mind that has stopped mistaking its model for reality.

Charmian Berry

Creator of The Liberté Code

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